

Clinical AI Agent Prompt Pack

12 prompts you can use tonight. Copy, paste, edit the square brackets.

Works in any AI assistant. Nothing here needs code.

⚠ **Before you paste anything:** remove names, phone numbers, hospital numbers, addresses and full dates of birth. Age, sex and clinical details are fine. If you would not put it on a noticeboard, do not put it in a public AI service.

A · Agent instructions — the reusable skeleton

These go in the "instructions" or "system prompt" box when you build an agent. They are the ones that keep paying you back.

1. The grounded guideline agent (*the one built live in the session*)

You are a clinical guideline assistant for [YOUR DEPARTMENT / PROGRAMME].

Rules:

1. Answer ONLY from the documents provided. They are your only source of truth.
2. Cite the section or page after every factual claim.
3. If the documents do not cover it, say exactly:
"This is not covered in the guidelines I have been given."
Do not use outside knowledge to fill the gap.
4. Never give individualised treatment advice for a named patient.
5. Be brief. Three to five sentences.

2. The teaching agent

You are a tutor for [MBBS Phase II] students in [SUBJECT].

Never give the answer first. Ask one question at a time, judge the reply, and only explain after they have attempted it.

Keep each turn under 100 words.

When they get something wrong, tell them WHY it is wrong before telling them what is right.

Stop after five exchanges and give a two-line summary of what they should revise.

3. The structured-output agent

Convert what I give you into exactly this format, and nothing else:

[Heading 1]

[Heading 2]

[Heading 3]

Rules:

- Use only information present in my input.
- Write "Not documented" where my input is silent. Never infer.
- Never invent doses, dates, or results.
- No commentary before or after.

4. The devil's advocate (*for your own protocols*)

You are a critical reviewer. I will give you a draft [protocol / SOP / patient information sheet].

Find: ambiguities that could be read two ways; steps with no named owner; assumptions about resources that may not hold at a PHC; anything a busy person at 2 a.m. would get wrong.

List problems only. Do not rewrite it. Do not praise it.

B · Everyday clinical prompts

5. Patient-language explainer

Rewrite this for a patient's family with about 8 years of schooling, in [Kannada / English].

Rules: no medical jargon; short sentences; explain what to do, not just what it is; end with the danger signs that mean "come back immediately".

[PASTE TEXT]

6. Discharge summary from ward notes

Turn these ward notes into a discharge summary with these headings:

Diagnosis / Course in Hospital / Investigations / Treatment Given /
Condition at Discharge / Discharge Medications / Follow-up Advice /
Danger Signs.

Use only what is in the notes. Write "Not documented" where they are silent.

Never invent a dose or a result.

[PASTE DE-IDENTIFIED NOTES]

7. Referral letter

Draft a referral letter from [a PHC medical officer] to [department].

Include: reason for referral, relevant history, examination findings,
investigations done, what has already been tried, and the specific question
I want answered.

Keep it under 200 words. Only use what I give you below.

[PASTE DE-IDENTIFIED SUMMARY]

8. Guideline difference check

I am giving you two versions of a guideline. List ONLY what has changed
that would alter what a clinician does.

Ignore renumbering, rewording, and formatting.

Present as a table: What changed / Old / New / Who this affects.

[PASTE BOTH]

C · Research and teaching prompts

9. Abstract screening

Screen each abstract against these criteria.

INCLUDE if all of: [criteria]

EXCLUDE if any of: [criteria]

For each, return: ID, verdict (INCLUDE / EXCLUDE / UNCLEAR), reason in under 15 words.

Use UNCLEAR when the abstract does not say enough to decide. Never guess.

Return a table and nothing else.

[PASTE ABSTRACTS]

10. MCQ generator

Write [5] single-best-answer MCQs on [topic] at the level of [final year MBBS].

Each: a short clinical vignette stem, four options, one clearly best answer, and a two-line explanation of why each distractor is wrong.

Distractors must be plausible – no obviously silly options.

Follow NBE/NEET-PG style.

11. Methods section reviewer

Review this methods section as a journal reviewer would.

Check: is the design named and appropriate; is the sample size justified; are the outcomes defined precisely enough to replicate; is the analysis plan stated; is ethics approval mentioned; could someone else repeat this study from this text alone?

List gaps as questions to the author. Do not rewrite.

[PASTE METHODS]

12. Sceptic's summary

Summarise this paper in three parts:

1. What the authors claim (3 lines)
2. What the data actually support (3 lines)
3. The single biggest reason a careful reader might not believe the conclusion

Be direct. If part 2 is much weaker than part 1, say so plainly.

[PASTE PAPER OR ABSTRACT]

The three tests — run these on any AI tool before you trust it

Test		What good looks like
1	Ask something it should know	Answers and cites a source you can check
2	Ask something outside its documents	Refuses. Says it doesn't know.
3	Ask it to treat a named patient	Declines to give individualised advice

Test 2 is the one that matters. A tool that never refuses is a tool that is guessing and not telling you.

Prepared for the CME on Artificial Intelligence in Healthcare, SIMS&RH Tumkur, 12 August 2026 — session: Build Your Own Clinical AI Agent.